Dignity of Risk

Medication Self-Administration & Insulin Refusal

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EFFECTIVE DATE	3 August 2026	**REVIEW DATE**	3 August 2027
PARTICIPANT	John Eric Ross	**NDIS NUMBER**	574 479 496
APPROVED BY	Kevin Rae Geronimo — General Manager	**STATUS**	Active

1. PURPOSE

This document addresses specific and recurring dignity of risk matters relating to **John Eric Ross's** medication behaviour. It covers two identified behaviours: (1) John requesting that oral medications be left near him rather than taking them immediately, and (2) John refusing insulin administration. It provides support workers with clear guidelines on how to manage both situations.

Dignity of risk recognises that all people, including those with disability, have the right to make choices about their own lives — even when those choices involve a degree of risk. However, this right must be balanced against **duty of care** obligations and the clinical realities of John's medical conditions. This document is guided by the **NDIS Practice Standards**, the **NDIS Code of Conduct**, and the **UN Convention on the Rights of Persons with Disabilities (UNCRPD)**.

2. PARTICIPANT OVERVIEW

Full Name	John Eric Ross
Date of Birth	30 December 1961 (Age 64)
NDIS Number	574 479 496
Primary Disability	Severe Diabetic Peripheral Neuropathy — sensorimotor impairment, reduced limb function, chronic pain
Key Medical	Type 2 Diabetes Mellitus | End Stage Kidney Disease (haemodialysis 3x/week) | SPC in situ
Decision-Making	John is cognitively capable of making his own informed decisions.
NOK	Beverly Ross (Wife) — 0416 104 301
GP	Dr Shahzad Ahmed — Plenty Valley Medical Centre — 03 9436 9677
Pharmacy	Terry White Pharmacy, South Morang — 03 9408 3222

3. IDENTIFIED BEHAVIOUR

John receives the majority of his medications via **Webster packs**, which are prepared by Terry White Pharmacy (South Morang) and administered by support workers. John requires assistance with medication administration due to his reduced hand function and impaired dexterity caused by severe peripheral neuropathy.

Behaviour A — Oral Medication Self-Administration:

- When support workers present John with his medications from the Webster pack, he frequently **requests that the medications be left near him** (e.g., on his bedside table, on his tray) rather than taking them immediately.
- This results in **significant delays** in medication intake. On multiple occasions, John has **forgotten to take the medications entirely** after they were left with him.
- When support workers attempt to **follow up or remind** John to take his medications, he **becomes visibly angry and agitated**. He may respond with raised voice, verbal refusal, or expressions of frustration directed at the support worker.

Behaviour B — Insulin Refusal:

- When support workers attempt to administer John's insulin (Novorapid / Optisulin), he frequently **refuses outright** — declining the injection entirely, in the same manner he refuses his other medications.
- This is **not** a case of John asking staff to leave insulin nearby. John **actively refuses** to have his insulin administered.
- John also frequently **refuses BGL checks**, which are required to determine the correct insulin dosage and monitor his blood glucose levels.
- When support workers attempt to **explain the importance of insulin or encourage** John to accept it, he **becomes visibly angry and agitated**.

4. RISK ANALYSIS — WHY THIS MATTERS

John's medications are clinically essential. Delayed or missed doses carry the following risks:

Medication	Risk if Delayed or Missed	Severity
Oral Medications (Webster pack)	Varies by medication. May include uncontrolled blood pressure, unmanaged pain, gastrointestinal complications, or worsening of chronic conditions. Cumulative missed doses compound risk over time.	**MEDIUM to HIGH**
Insulin (Novorapid / Optisulin)	Hyperglycaemia (dangerously high blood sugar). Risk of diabetic ketoacidosis (DKA) — a life-threatening emergency requiring hospitalisation. Compounded by John's End Stage Kidney Disease. Repeated missed doses accelerate kidney damage and increase risk of cardiac events, infection, and poor wound healing.	**HIGH — CRITICAL**
BGL Monitoring (3x daily)	Without BGL readings, staff cannot determine safe insulin dosage. Risk of administering incorrect dose if BGL is unknown. Risk of undetected hypoglycaemia or hyperglycaemia.	**HIGH**

5. STAFF GUIDELINES — HOW TO RESPOND

The following step-by-step guidelines must be followed by all support workers when John requests that his medications be left near him. These guidelines balance John's dignity of risk with the clinical requirements of his care.

For Behaviour A — Oral Medications (Webster Pack):

STEP 1	**PRESENT MEDICATIONS AS NORMAL** Bring the Webster pack to John at the scheduled time. Offer water or juice. Calmly and respectfully ask John if he is ready to take his medications now.
STEP 2	**IF JOHN ASKS YOU TO LEAVE THEM — ACKNOWLEDGE AND INFORM** Do **not** argue or insist. Instead, acknowledge his request calmly. Inform John once, clearly and without pressure: *"That's okay John. I just want to let you know these are your [morning/lunch/evening] medications and they work best when taken on time. I'll leave them here for you."* Do not lecture, repeat the risks multiple times, or adopt a parental tone.
STEP 3	**LEAVE THE MEDICATIONS IN A SAFE, VISIBLE LOCATION** Place the medications within John's reach and line of sight (e.g., bedside table, tray table). Ensure the Webster pack compartment is clearly labelled for the correct day and time.
STEP 4	**SET A 30-MINUTE CHECK-IN (ONE FOLLOW-UP ONLY)** Return after **30 minutes** to check whether John has taken his oral medications. Approach calmly: *"Hey John, just checking in — did you get a chance to take your tablets?"* If John has taken them, document the delay and move on. If John has not taken them, note this and proceed to Step 5. **Do not ask more than once.** Repeated prompting triggers anger and is counterproductive.
STEP 5	**IF MEDICATIONS REMAIN UNTAKEN AFTER 30 MINUTES** Do **not** continue to prompt John. Instead: • Document in shift notes: time offered, John's request to leave them, 30-minute follow-up, and that medications remain untaken. • Notify the **Team Leader** via phone or message. • Do not remove the medications from John's reach unless he asks you to.
STEP 6	**IF JOHN BECOMES ANGRY OR AGITATED** If at any point John becomes angry when you follow up: • Do not escalate. Do not raise your voice or argue. • Calmly say: *"No worries John, I'll leave you to it."* • Remove yourself from the situation. Give John space. • Document the interaction in shift notes, including John's response. • Notify Team Leader. Do not take it personally — this is a known behavioural pattern and is not a reflection of your care.

For Behaviour B — Insulin Refusal:

STEP 1	**OFFER BGL CHECK & INSULIN AS NORMAL** Approach John at the scheduled time with BGL monitor and insulin. Calmly and respectfully ask John if he is ready for his BGL check and insulin.
STEP 2	**IF JOHN REFUSES — ACKNOWLEDGE AND INFORM ONCE** Do **not** argue or insist. Inform John once, clearly: *"That's okay John. I just want to let you know your insulin is important for keeping your sugar levels safe. If you change your mind, let me know."*
STEP 3	**OFFER AGAIN AFTER 30 MINUTES (ONE FOLLOW-UP ONLY)** Return after **30 minutes**: *"Hey John, are you happy to have your insulin now?"* If he accepts, do BGL check and administer. If he refuses again, proceed to Step 4.
STEP 4	**IF JOHN STILL REFUSES** Do **not** continue to prompt. Document in shift notes. Notify Team Leader. Monitor for signs of hyperglycaemia (excessive thirst, frequent urination, confusion, nausea, fruity breath). If John refuses BGL check, do **not** administer insulin without a reading.
STEP 5	**MONITOR FOR SYMPTOMS** Even if John refuses, continue observing for deterioration. If confusion, vomiting, or rapid decline is observed, contact Team Leader and consider calling 000.

■ **REMEMBER: Oral medications (Webster pack) may be left with John as per his request. Insulin is different — John refuses it outright. In both cases: offer once, follow up once at 30 minutes, then document and notify Team Leader. Do not force or repeatedly prompt.**

6. WHAT STAFF MUST NOT DO

✘ **Do not force or coerce** John to take his medications or insulin. This violates his rights under the NDIS Code of Conduct and may constitute restrictive practice.

✘ **Do not repeatedly prompt or nag.** One initial offer and one 30-minute follow-up is the maximum. Repeated prompting causes anger, damages rapport, and is ineffective.

✘ **Do not withhold other supports** (e.g., meals, assistance) as a consequence of John not taking his medications or refusing insulin.

✘ **Do not make John feel guilty** or use emotional pressure (e.g., "You'll end up in hospital if you don't take these"). State facts once, then respect his decision.

✘ **Do not ignore the situation.** Even if John declines, documentation and Team Leader notification are mandatory.

✘ **Do not administer insulin without a BGL reading.** If John refuses BGL check, insulin cannot be safely administered — document and notify Team Leader.

7. DOCUMENTATION REQUIREMENTS

Every instance of delayed or declined medication must be documented in the shift notes. Include:

- Date and time medications were offered
- John's response (e.g., "John asked for tablets to be left on his bedside table")
- Time of 30-minute follow-up and outcome (taken / not taken)
- Whether John became agitated and how it was managed
- Whether Team Leader was notified (and their response)
- Support worker's name

Example shift note entry:

"08:15 — Offered John his morning BGL check and insulin. John refused, saying he didn't want it. Informed John calmly that insulin is important for keeping his sugar levels safe. John said 'Not now.' Did not press further. 08:45 — Returned and offered again. John refused again in a firm tone. Did not prompt further. Notified TL [name] via text at 08:50. Monitoring John for symptoms — no signs of hyperglycaemia observed during shift. — [Staff Name]"

8. ESCALATION PROCESS

Situation	Action
John initially refuses but accepts insulin within 30 minutes	Administer insulin. Document delay in shift notes. No further escalation required unless pattern is increasing.
John refuses insulin entirely for that dose	Document in shift notes. Notify Team Leader. Monitor for symptoms. Do not continue to prompt John.
John refuses BGL check (so insulin cannot be safely given)	Document refusal. Do not administer insulin without BGL. Notify Team Leader. Monitor for symptoms.
John becomes aggressive or threatening when offered insulin	Remove yourself. Ensure your safety. Document. Notify Team Leader and On-Call Manager. Complete incident report if required.
Repeated pattern of refused insulin (3+ instances in a week)	Team Leader to escalate to management. Case conference with GP (Dr Ahmed), Support Coordinator (Gulsun), and Beverly (NOK). Review insulin schedule and approach.

9. QUICK REFERENCE SUMMARY

Can I leave Webster pack tablets with John?	**YES** — Place within reach. Follow up once after 30 mins. Document.
Can John refuse insulin?	**YES** — It is his right. Offer once, follow up once at 30 mins. Document and notify TL.
What if John refuses BGL check?	Do **not** give insulin without BGL. Document refusal. Notify TL. Monitor for symptoms.
How many times can I remind/offer?	**Once** at initial offer + **one** follow-up at 30 minutes. Maximum two interactions.
What if John gets angry?	Back off. Say *"No worries John."* Do not escalate. Document and notify TL.
Is this my fault?	**No.** As long as you followed these steps and documented, you have met your duty of care. John has the right to make these choices.

10. PARTICIPANT ACKNOWLEDGEMENT

I, **John Eric Ross**, acknowledge and understand the following:

• I have been informed about the importance of taking my medications on time, including my Webster pack medications and insulin, and I understand the medical risks associated with delayed or missed doses.

• I understand that when I choose to delay taking my medications, ask staff to leave them near me, or refuse my insulin, this is **my decision** and I accept the risks associated with those choices.

• I understand that support workers are required to offer my medications and insulin at the scheduled time and may follow up with me **once** after 30 minutes. I understand this is part of their duty of care and is not intended to pressure or disrespect me.

• I understand that if I decline my medications or refuse insulin, staff are required to document this and notify their Team Leader. This is a standard procedure and not a punitive measure.

• I have had the opportunity to ask questions about this document and have had them answered to my satisfaction.

Participant Name	John Eric Ross	**Date**	
Participant Signature		**Witness Name**	
NOK Name	Beverly Ross	**NOK Signature**	

11. STAFF ACKNOWLEDGEMENT

By signing below, I confirm that I have read and understood the Dignity of Risk guidelines specific to John Eric Ross's medication self-administration and insulin refusal behaviours. I understand that oral medications may be left with John per his request, and that John has the right to refuse insulin. I will follow the step-by-step processes, limit follow-ups to one, and document all instances.

Name	Signature	Date	Initials